

Heme/Onc Literature Surveillance -- Cycle 2026-07-03

- **Surveillance window:** 2026-06-19 to 2026-07-03
 - **Disease groups covered:** Diffuse Large B-Cell Lymphoma
 - **Total new items surfaced:** 3
 - **Flagged for retraction/concern:** 0
 - **Needs human review:** 0
 - **Recent, not yet indexed:** 2 Tier 1, 3 Tier 2 — reported separately in the `recent_tier1` / `recent_tier2` files (identification info + abstract; not AI-appraised).
 - **Mode:** abstracts only — no AI summarization, extraction, or appraisal. Each item shows its identification info and the verbatim source abstract.
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Diffuse Large B-Cell Lymphoma

Trials

Ten-Year Outcomes after CAR T-Cell Therapy for B-Cell Lymphomas.

Journal: The New England journal of medicine | **Date:** 2026-Jun-25 | **PMID:** 42341302 | **DOI:** 10.1056/NEJMoa2518035

Publication types: Clinical Trial, Phase II, Journal Article, Randomized Controlled Trial

Abstract:

BACKGROUND: Anti-CD19 chimeric antigen receptor (CAR) T-cell therapy is a standard treatment for relapsed or refractory B-cell non-Hodgkin lymphomas. Long-term results and curative potential remain uncertain.

METHODS: We evaluated long-term outcomes in 38 patients with relapsed or refractory B-cell non-Hodgkin lymphomas (24 patients with large B-cell lymphoma and 14 with follicular lymphoma) who had been treated with CTL019 (now called tisagenlecleucel) - autologous T cells expressing CD19-directed, 4-1BB-costimulated chimeric receptors.

Lymphoma-free survival was defined as the time from the tisagenlecleucel infusion to relapse or lymphoma-related death. The incidence of non-relapse-related death and second primary cancer was estimated with the Aalen-Johansen method. The data-cutoff date was October 1, 2025.

RESULTS: At a median follow-up of 10.1 years (range, 7.9 to 11.5), no relapses had occurred beyond 5.4 years. The 10-year lymphoma-free survival was 32% (95% confidence interval [CI], 14 to 51) among patients with large B-cell lymphoma and 47% (95% CI, 20 to 71) among those with follicular lymphoma. In an analysis that included deaths from any cause, the 10-year progression-free survival was 17% (95% CI, 5 to 34) among patients with large B-cell lymphoma and 29% (95% CI, 9 to 52) among those with follicular lymphoma; the 10-year overall survival was 17% (95% CI, 5 to 34) and 50% (95% CI, 23 to 72), respectively. Persistent grade 2 or 3 neutropenia occurred in 2 patients (5%); no late anemia or thrombocytopenia was observed. A second primary cancer developed in 9 patients (10-year cumulative incidence, 21%). The 10-year non-relapse-related mortality was 18% (14% when deaths related to coronavirus disease 2019 were excluded). Higher CAR-transgene persistence appeared to be associated with long-term response. B-cell aplasia persisted in 44% of patients with a long-term response.

CONCLUSIONS: Among patients with heavily pretreated B-cell non-Hodgkin lymphoma, a single infusion of tisagenlecleucel led to decade-long remissions (lymphoma-free survival) in approximately one third of the patients with large B-cell lymphomas and in nearly one half of those with follicular lymphoma. (Funded by the Richard Berman Family Innovations Center in CLL and Lymphomas and others; ClinicalTrials.gov number, NCT02030834.).

Tafasitamab plus lenalidomide and R-CHOP versus R-CHOP for first-line treatment of patients with high-risk diffuse large B-cell lymphoma (frontMIND): a global, phase 3, randomised, double-blind, placebo-controlled trial.

Journal: Lancet (London, England) | **Date:** 2026-Jun-20 | **PMID:** 42217458 | **DOI:** 10.1016/S0140-6736(26)00866-4

Publication types: Journal Article, Randomized Controlled Trial, Clinical Trial, Phase III, Multicenter Study

Abstract:

BACKGROUND: Approximately 40% of patients with high-risk diffuse large B-cell lymphoma (DLBCL) are not cured with first-line R-CHOP (rituximab, cyclophosphamide, doxorubicin, vincristine, and prednisone or prednisolone). We aimed to investigate the addition of tafasitamab (an Fc-enhanced anti-CD19 monoclonal antibody) and lenalidomide to R-CHOP (tafa-len-R-CHOP) in patients with high-risk aggressive B-cell lymphomas.

METHODS: frontMIND is a phase 3, randomised, double-blind, placebo-controlled study conducted at 298 centres in North America, South America, Europe, and the Asia-Pacific region. Patients aged 18-80 years with previously untreated, high-intermediate-risk or high-risk DLBCL or high-grade B-cell lymphoma (HGBL) were randomly allocated (1:1), stratified by International Prognostic Index (IPI) or age-adjusted IPI and geographical region, to receive six 21-day cycles of standard R-CHOP (rituximab 375 mg/m

FINDINGS: Between May 11, 2021, and March 2, 2023, 1229 patients were screened, among whom 899 were randomly allocated: 448 (50%) to the tafa-len-R-CHOP group and 451 (50%) to the R-CHOP group. At the time of primary analysis (median follow-up 35.2 months [95% CI 35.0-35.4]), progression-free survival was improved in the tafa-len-R-CHOP group versus the R-CHOP group (hazard ratio [HR] 0.75 [95% CI 0.59-0.96]; $p=0.0194$), with 2-year progression-free survival rates of 71.1% (66.3-75.4) with tafa-len-R-CHOP versus 62.9% (57.9-67.5) with R-CHOP. Interim HR for overall survival was 0.85 (0.63-1.14). The overall rate of grade 3 or higher treatment-emergent adverse events was higher with tafa-len-R-CHOP (384 [87%] of 443) than with R-CHOP (340 [76%] of 447). Additionally, a higher rate of fatal treatment-emergent adverse events was observed with tafa-len-R-CHOP (26 [6%]) than with R-CHOP (17 [4%]).

However, the number of overall deaths in the study was lower with tafa-len-R-CHOP than with R-CHOP (82 [19%] vs 97 [22%]). Based on disposition data, rates of premature discontinuation of all study drugs were similar in the tafa-len-R-CHOP group (71 [16%] of 443) and R-CHOP group (66 [15%] of 447).

INTERPRETATION: Progression-free survival was significantly improved with tafa-len-R-CHOP versus R-CHOP; however, the safety profile indicated increases in adverse events, including treatment-emergent adverse events leading to death, with the addition of tafasitamab and lenalidomide. Overall survival data are immature; follow-up is ongoing. Further analyses, including of circulating tumor DNA, will help to assess whether deeper molecular responses are contributing to the progression-free survival benefit observed with tafa-len-R-CHOP. Tafa-len-R-CHOP might represent a potential new first-line treatment for patients with high-risk DLBCL or HGBL.

FUNDING: Incyte Corporation.

Epcoritamab monotherapy or epcoritamab with lenalidomide as first-line therapy for patients with diffuse large B-cell lymphoma (EPCORE DLBCL-3): primary analysis of an open-label, multicentre, randomised, phase 2 trial.

Journal: The Lancet. Haematology | **Date:** 2026-Jul | **PMID:** 42285113 | **DOI:** 10.1016/S2352-3026(26)00112-2

Publication types: Journal Article, Randomized Controlled Trial, Multicenter Study, Clinical Trial, Phase II

Abstract:

BACKGROUND: Anthracycline-free regimens are needed for older adults with newly diagnosed diffuse large B-cell lymphoma (DLBCL). We aim to evaluate the efficacy and safety of fixed-duration epcoritamab monotherapy versus epcoritamab with lenalidomide in this patient population.

METHODS: This open-label, multicentre, randomised, phase 2 trial was conducted at 44 hospitals across 11 countries in Europe and Asia. Patients had newly diagnosed, histologically confirmed CD20-positive large B-cell lymphoma, were ineligible for anthracycline-based chemoimmunotherapy (age ≥ 80 or ≥ 75 years with clinically significant comorbidities), had Ann Arbor stage II-IV disease, and an Eastern Cooperative Oncology Group (ECOG) performance status of 0-2. In stage 1, patients were randomly assigned (1:1) to epcoritamab monotherapy or epcoritamab with lenalidomide using a

validated interactive response technology system and stratified by the International Prognostic Index (<3 vs ≥3) and ECOG performance status (0-1 vs 2). Epcoritamab was administered subcutaneously using a two step-up dosing schedule (0·16 mg on cycle 1 day 1 and 0·8 mg on cycle 1 day 8), followed by full 48 mg doses once per week in 28-day cycles during cycles 1-3 and once every 4 weeks during cycles 4-12 for up to 12 cycles. Lenalidomide (10 or 20 mg) was given orally once a day on days 1-21 of 28-day cycles for up to 12 cycles. Based on stage 1, one of the treatment regimens was selected for expansion in stage 2. The primary endpoint was investigator-assessed complete response rate (as of the data cutoff on Dec 5, 2025; Lugano criteria) in the full analysis set (all randomly assigned patients in stage 1 and in all treated patients in stage 2). Safety was analysed in patients who received ≥1 dose of trial treatment. This study is registered with ClinicalTrials.gov, NCT05660967 (active, not recruiting).

FINDINGS: Between March 8, 2023, and May 14, 2025, 111 patients were assessed for eligibility (88 in stage 1 and 23 in stage 2) and 108 were treated. Median age was 83·0 years (IQR 80·0-86·0). 51 (47%) of 108 were male, 57 (53%) were female, and 87 (81%) were White. In stage 1, 44 patients each were randomly assigned to epcoritamab monotherapy or epcoritamab with lenalidomide and two did not receive treatment. At data cutoff, the complete response rate in stage 1 was 63·6% (95% CI 47·8-77·6; in 28 of 44 patients) in the epcoritamab monotherapy group and 45·5% (30·4-61·2; in 20 of 44) in the epcoritamab with lenalidomide group. The most common treatment-emergent adverse events (grade ≥3) were infections (eight [18%] of 44 patients) and fatigue (six [14%]) with epcoritamab monotherapy versus neutropenia (17 [40%] of 42) and infections (16 [38%]) with epcoritamab with lenalidomide; with serious adverse events in 28 (64%) versus 34 (81%). Treatment-emergent deaths occurred in six (14%) patients in the monotherapy group (cytomegalovirus infection reactivation, COVID-19 pneumonia, SARS-COV-2 infection, tumour lysis syndrome, neuroendocrine tumour of the lung, tumour haemorrhage) and six (14%) in the combined group (pneumonia, COVID-19 pneumonia, sepsis, general physical health deterioration, multiple organ dysfunction syndrome, acute cardiac failure). Based on differences in complete response rates and safety, epcoritamab monotherapy was selected for stage 2 and one patient did not receive treatment. In the epcoritamab monotherapy group, the complete response rate was 45·5% (24·4-67·8; in ten of 22) in stage 2 and 57·6% (44·8-69·7; in 38 of 66) across stages 1 and 2. The most common treatment-emergent adverse events (grade ≥3) among all patients who received epcoritamab monotherapy were infections (16 [24%]), neutropenia (eight [12%]), and hypertension (seven [11%]); with serious adverse events in 46 (70%). Treatment-emergent deaths occurred in two (3%) patients in stage 2 (multiple organ dysfunction syndrome and acute respiratory failure).

INTERPRETATION: Fixed-duration epcoritamab monotherapy showed promising complete response rates and a manageable safety profile in older adults with newly diagnosed DLBCL and comorbidities, with slight differences between stages 1 and 2. Continued investigation of epcoritamab as a first-line chemotherapy-free treatment option is warranted.

FUNDING: Genmab and AbbVie.

Needs human review

None this cycle.

Flagged: retraction / expression of concern

None this cycle.

Footer: exact queries used

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